SWASTHYA DIAGNOSTICS
DEFINING HEALTH
A Unit of kidney Care Centre, Panchkula

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UHID No	:-	UD-24250009812	**Reg No**	:-	24250009812
Patient Name	:-	Mr VIPIN KUMAR GARG	**Sample Gnt At**	:-	27 Aug 2025 08:59 AM
Age/Sex	:-	67 Y/M	**Sample Rvd At**	:-	27 Aug 2025 08:59 AM
Address	:-	323, SECTOR 9 PANCHKULA , Panchkula	**Report Gnt At**	:-	27 Aug 2025 10:12 AM
Mobile No	:-	9815311118	**Consultant**	:-	Dr AJAY GOYAL (DM(NEPHROLOGY))

Final Report

Investigations	Observations	Biological Ref. Interval	Unit
BIOCHEMISTRY			
KIDNEY FUNCTION TEST (KFT)			
UREA, SERUM *(Serum, Urease)*	38.6	8.90 - 40.00	mg/dl
CREATININE, SERUM *Enzymatic*	**1.83**	0.50 - 1.30	mg/dl
URIC ACID, SERUM *Uricase*	**8.8**	3.50 - 7.20	mg/dl
BLOOD UREA NITROGEN(BUN)	18.04	6.00 - 21.00	mg/dl
SODIUM,SERUM *ISE*	136.0	136.00 - 145.00	mEq/L
POTASSIUM, SERUM *ISE*	**5.70**	3.50 - 5.50	meq/L
CHLORIDE, SERUM *ISE*	105.0	96.00 - 106.00	meq/L
SEROLOGY			
VIRAL MARKERS			
HIV 1 & 2 ANTIBODY SCREENING TEST, SERUM	**Non-Reactive**	Non Reactive	
HEPATITIS C ANTIBODY (ANTI HCV), SERUM	**Non-Reactive**	Non-Reactive	
HEPATITIS B SURFACE ANTIGEN (HBSAG), SERUM *IMMUNOCHROMATOGRAPHY CARD METHOD*	**Non-Reactive**	Non-Reactive	

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Note: 1. All Reactive results are tested additionally by Specific antibody Neutralization assay . For further confirmation Molecular assays are recommended
2. Discrepant results may be observed during pregnancy, patients receiving mouse monoclonal antibodies for diagnosis or therapy & mutant forms of HBsAg
3. For diagnostic purposes, results should be used in conjunction with clinical history and other hepatitis markers for Acute or Chronic infection
4. For monitoring HBsAg, anti HCV levels, Quantitative HBsAg and antibody to HCV assay is recommended
Uses - Routine screening of blood and blood products to prevent transmission of Hepatitis B virus (HBV) to recipients, To diagnose suspected HBV, HCV and HIV infection and monitor the status of infected individuals, To evaluate the efficacy of antiviral drugs, For Prenatal Screening of pregnant women.

Remarks :-

URINE EXAMINATION

URINE ROUTINE EXAMINATION

PHYSICAL EXAMINATION

COLOR	PALE YELLOW	PALE YELLOW	
APPEARANCE	CLEAR	CLEAR	
PH(REACTION)	6.0	4.50 - 7.00	
SPECIFIC GRAVITY	1.020	1.000-1.030	

CHEMICAL EXAMINATION

GLUCOSE	Absent	Absent	
URINE PROTEIN	Absent	Absent	
KETONE	Absent	Absent	
UROBILINOGEN	Normal	Normal	
BILE PIGMENTS	Absent	Absent	
BILE SALTS	Absent	Absent	
NITRITE	Absent	Absent	

MICROSCOPIC EXAMINATION

PUS CELL	2-3	0.00 - 5.00	/HPF
URINE RBC	Absent		/HPF
EPITHELIAL CELLS	1-2		/HPF

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CASTS	Absent	Absent
CRYSTALS	Absent	Absent
BACTERIA	Absent	Absent

BIOCHEMISTRY

URINE ALBUMIN CREATININE RATIO (UACR), SPOT URINE

ALBUMIN, URINE	8.95		mg/L
CREATININE, URINE *Enzymatic*	158.33		mg/dl
ALBUMIN : CREATININE RATIO *Calculated*	5.65	0.00 - 30.00	mg/g creatinine

CATEGORY	REFERENCE RANGE IN mg/g CREATININE
Normal / Non diabetic	< 30
Microalbuminuria	30-300
Clinical albuminuria	> 300

Note:1. Patient is considered to be within diagnostic category if atleast 2 out of 3 specimens collected within a period of 3-6 months show abnormal results.
2.This test is useful in the management of patients with early Diabetes mellitus to assist in avoiding or delaying the onset of renal disease. Due to inherent day to day variability in albumin excretion, this ratio is a better indicator than isolated micro albumin levels in urine.

Remarks :-

* END OF REPORT *

(Technologist)

Dr. Kavita Goyal
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HMC Reg. No. 14871

Note :- * If the result of the test is alarming or unexpected,the patient is contact the laboratory immediately
* The lab report is not the diagnosis,it represents only an opinion. Kindly Contact your doctor for interpretation diagnosis and treatment. Not Valid for Medico legal purpose *Report is Electronically validated.